

Response and Proposed Allocation of Questions

Request for Medical Information dated 31 July 2026

Employee: Cory Lea Shepherd, AO3, Switchboard Services, Logan Hospital

Reference: MSH-INJ-5795 · CLM-317073

Request issued by: Human Resources, Logan and Beaudesert Health Service — 31 July 2026

Response date: 3 August 2026

IN SUMMARY

1. I am engaging with this process and will obtain and provide the information sought.
2. Four of the nine questions are not medical questions at all. Questions 2, 3, 6 and 9 ask my doctor to answer matters which the Health Service's own policy and the Work Health and Safety Act 2011 place with the Health Service. I ask that it answer those itself, after which the properly medical part of each can be put to my practitioners.
3. Of the remainder, Questions 1(c), 1(d), 4, 5 and 8 are for my general practitioner, and Questions 1(a), 1(d), 7 and 8 require or are better answered by my treating psychiatrist. I will obtain all of them. Dr Ma's understanding of my psychiatric condition derives from my psychiatrist's correspondence to him, which is why several questions are better put to the specialist directly — and is the answer to Question 1(c).
4. Question 1(a) I answer openly in this document. Question 1(b) alone I ask be withdrawn: it asks a general practitioner for a psychiatric causal formulation, and it goes to a matter in issue in WC/2024/227.
5. I ask to be returned to my roster now. The restriction in issue asks that complaints be redirected rather than absorbed — which is the process the Health Service says it already operates, and which the Role Description does not require of me in any event.
6. I ask for an extension of the seven days, and explain at Part 7 why that period cannot be met on the Health Service's own question set.
7. I ask who the delegate is, and propose at Part 6.1 how the conflict can be managed: the medical information held exclusively by Injury Management, the decision made outside Corporate Services, and implementation retained within it.
8. My proposal for returning to work, and the confirmation of my substantive position, are in the accompanying Proposal for Return to Work. Part 9 lists what I ask the Health Service to confirm.

PART 1 — THE ORIGIN OF THE REQUEST, AND THE TEST APPLIED

1.1 THE ORIGIN AND STATED PURPOSE OF THE EMPLOYEE CAPABILITIES CHECKLIST

The letter of 31 July 2026 records the origin of the checklist in the Health Service's own words:

"your recently completed Employee Capabilities Checklist (ECC) (Attachment 1) completed by Dr Day Hong Ma on the 3 July 2026 which was provided to us to facilitate a Return to Work Plan."

That is correct, and the sequence was as follows.

- (a) On 5 May 2026 the Manager, Switchboard Services asked me to arrange for the Employee Capabilities Checklist to be completed, so that the Health Service could "provide the most appropriate support in the workplace" and "discuss the next steps to ensure you are supported and working safely". No condition was attached to my attendance.
- (b) On 21 May 2026 I advised that an appointment was booked for 5 June 2026. In the same email I arranged a swap for a shift I was rostered on the following day. I remained rostered and at work with the checklist outstanding, and no suggestion was made that I could not attend.
- (c) On 26 June 2026 the position changed. I was advised that "until we receive appropriate medical clearance, including a completed Employee Capacity Certificate (ECC), we are unable to facilitate your return to work", by reference to section 19 of the Work Health and Safety Act 2011, and that without the documentation I "will be unable to return to work".
- (d) That position was repeated on 2 July 2026, with a leave-type deadline of 2:00pm on Friday 3 July 2026.
- (e) I obtained the checklist and provided it on 3 July 2026 at 2:32pm. It certified me fit to work with restrictions.

(f) At 3:18pm on 3 July 2026 I asked the Health Service to confirm whether I was to attend, or otherwise to advise my next returning shift. That request has not been answered. My rostered shifts were charged to accrued leave, then to sick leave without pay, and from 13 July 2026 to income protection without pay. I have not worked since.

The checklist was therefore required by the Health Service, for a purpose the Health Service has itself stated — to facilitate a return to work — and, once provided, became the basis on which work was refused. It did not certify me unfit. It certified me fit with restrictions.

I record this because it bears directly on the present request. The Health Service now seeks further information about a document it required me to obtain in order to work, and which certified that I could.

I am willing to provide clarification of that document. I ask that the clarification be directed to identifying the adjustments which allow me to work — the purpose for which the document was obtained, the purpose stated in the letter of 31 July 2026, and the purpose contemplated by Human Resources Policy G03.

1.2 THE TEST APPLIED

The letter to Dr Ma states the purpose of the request:

"to identify and implement any appropriate workplace controls or reasonable adjustments to support a safe and sustainable return to his substantive position."

Human Resources Policy G03 (QH-POL-210), which the Health Service has cited, sets the limit at clause 3:

"Reasonable adjustment may need to be supported by medical or rehabilitation information, where necessary, to either identify required workplace adjustments and/or assess the suitability of proposed adjustments."

I have applied that test to each question: is this information necessary to identify a required adjustment, or to assess the suitability of a proposed one — and is the person being asked the right person to answer it?

Applying that test, the nine questions divide four ways. I am not declining to provide information. I am proposing that each question be directed to whoever is able to answer it.

1.3 THE CHECKLIST ALREADY ANSWERS MOST OF WHAT IS NOW ASKED

The Employee Capabilities Checklist is Attachment 1 to the Health Service's own letter. Read in full, it already answers, in substance, most of the nine questions.

Q1(d) foreseeable risk and necessary controls — answered at Additional Comments 3 and 4, which set out the recommended work pattern with clinical reasoning for each shift type, and five specific workplace modifications.

Q4 restrictions, clinical basis in functional terms, duration and review — answered at Additional Comments 2: "Episodic, with symptom exacerbation linked to stressor exposure. The restrictions below are expected to remain necessary for at least the next 3-6 months, subject to treatment response and 8-weekly review." The review date is given on the first page as 28 August 2026.

Q5 what "complaint handling" encompasses — answered: "complaints received at the Switchboard are to be logged and redirected to management/the dedicated complaints area, not actioned, resolved or absorbed by Mr Shepherd."

Q6 the requirements of the role — answered: "Usual switchboard operational duties remain suitable; complaint-handling duties are excluded as below."

Q7 working memory in functional terms — addressed under Memorising information ("Working memory affected under stress") and at Additional Comments 1 ("reduced concentration and working memory under sustained high-demand load"), with the roster controls which manage it.

Q8 tasks and environments which exacerbate the condition — answered at Additional Comments 1 and in the foreseeable-stressors response.

Q2 capacity to follow a direction — the checklist records "Understanding instructions: Normal" and "Carrying out instructions accurately: Normal."

I do not raise this to refuse further clarification. I have set out at Part 3 what my practitioners will provide. I raise it because I have been excluded from the workplace for five weeks while information the Health Service has held since 3 July 2026 is sought again.

1.4 THREE MATTERS ON THE FACE OF THE CHECKLIST

- (a) **It records a continuation, not a return.** The first page states: "Fit to commence the above from: 3 July 2026 (continuation of existing arrangement)."
- (b) **It records twelve months of sustained work.** At Additional Comments 3: "This reduced pattern of approximately six shifts per fortnight is the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration, and maintaining rather than increasing it best supports recovery while minimising risk of exacerbation."
- (c) **The restriction in issue asks for nothing the Health Service does not already do.** The checklist asks that complaints be "logged and redirected to management/the dedicated complaints area." The letter of 31 July states that the established process is "immediate escalation and management by the Health Service Client Liaison Officer and/or Manager, Switchboard Services." Those are the same arrangement. The Role Description at Attachment 2 contains no complaint-handling responsibility.

I record also that the checklist recommends, at Additional Comments 4(c), "return-to-work coordination and correspondence through the Injury Management team, Human Resources." That is the substance of what I propose in the accompanying Proposal for Return to Work.

PART 2 — PROPOSED ALLOCATION

Q	SUBJECT	PROPOSED RESPONDENT
1(a)	Date of first diagnosis	Answered openly; clinical context from the psychiatrist
1(b)	Clinical basis for the causal link	Withdrawal requested — a psychiatrist question, and in issue in the appeal
1(c)	Self-report v clinical assessment; other reports held	Consented to — Dr Ma. Answer: the specialist reports he holds, not self-report
1(d)	Foreseeable risk; controls medically necessary	Psychiatrist; Dr Ma from the specialist documentation he holds
2	Following directions; performance and conduct discussions	Health Service — threshold question
3	Fitness under the existing reporting arrangements	Health Service — risk assessment, then treating psychiatrist
4	Restrictions, adjustments, duration and review date	Dr Ma (GP)
5	Meaning of "complaint handling"	Health Service — already answered, then Dr Ma
6	Requirements of the role without modification	Health Service — identify genuine occupational requirements, then GP
7	"Working memory affected under stress"	Treating psychiatrist
8	Tasks and environments that exacerbate the condition	Psychiatrist; Dr Ma from the specialist documentation he holds
9	If unable to accommodate, whether safe return is possible	Health Service — onus is the employer's

Of the nine questions, four are not medical questions. They are questions the Health Service must answer itself before any clinician can usefully be asked anything.

This allocation applies to whatever question set the Health Service confirms. If questions are withdrawn, amended or added, I will allocate the confirmed set on the same basis — each medical question to whichever of my practitioners is the right person to answer it, and the questions at Part 4 to the Health Service.

PART 3 — QUESTIONS FOR MY PRACTITIONERS

Dr Ma completed the Checklist and can answer within general practice and from his own file. My psychiatric condition is managed by my treating psychiatrist, and Dr Ma's understanding of it derives from that specialist's correspondence to him — which is also, in substance, the answer to Question 1(c): his view of the identified stressors rests on the specialist reports he holds, not on my self-report alone.

3.1 FOR DR MA — I will obtain answers to these

Q1(d) foreseeable risk and the controls medically necessary — from the specialist documentation he holds; because the risk is psychiatric, I will also put it to my psychiatrist.

Q4 restrictions, clinical basis in functional terms, duration and review date.

Q5 what "complaint handling" encompasses as a clinical restriction — the operational meaning is dealt with at Part 4.3.

Q8 tasks, situations or environments which may exacerbate the condition — likewise also put to my psychiatrist.

Q1(c) I consent to this. Dr Ma can confirm what he holds and has relied on — his own file — and I ask only that the question not be read as an inquiry into material held by others.

The Checklist form states that "a separate medical certificate is also required". One was not provided on 3 July 2026; I will obtain it when I rebook with Dr Ma, or sooner if required.

3.2 FOR A TREATING PSYCHIATRIST — I am willing to obtain this

Q1(a) I answer this openly: the diagnosis was made on 24 October 2024. What makes a date meaningful is the clinical context, which is my psychiatrist's to give; and the question as framed asks a general practitioner to certify that no other diagnosis exists, which is beyond his scope. I ask that it be put in psychiatric terms.

Q1(d), Q8 as above — a specialist view of a psychiatric risk and its exacerbating factors.

Q7 what "working memory is affected under stress" means in functional terms — a functional cognitive opinion is a psychiatrist's, not a general practitioner's.

Please confirm which questions the Health Service presses. I will then instruct my psychiatrist, with the invoice directed to you consistent with the Health Service's confirmation that it will meet the reasonable costs. The opinion will be directed to current capacity, functional restrictions and recommended adjustments — the stated purpose of this inquiry — and not to the cause of my injury, which is in issue in WC/2024/227 and is not necessary for that purpose.

The sequence bears on the timeframe: I cannot instruct either practitioner until the question set is settled, and none of that can occur within seven calendar days. The only step which cannot presently begin is the one which depends on the Health Service. I note that a request to my own treating practitioners rests on a different footing from an independent medical examination under chapter 3 part 8 division 5 of the Public Sector Act 2022, which has not been invoked.

PART 4 — QUESTIONS FOR THE HEALTH SERVICE

These four questions ask a doctor to answer matters which the Health Service's own policy and the Work Health and Safety Act 2011 place with the Health Service. I ask that the Health Service answer them, after which the properly medical component of each can be put to my practitioners.

4.1 QUESTION 2 — is there a performance or conduct process?

The question asks whether I can "follow a reasonable and lawful direction issued by [my] supervisor" and "participate in discussions in relation to [my] workplace performance and/or conduct."

These are not questions about capacity to perform the genuine occupational requirements of my role. They concern readiness for a management process. Either such a process is on foot concerning me — in which case I am entitled to be told of it, given particulars, and afforded procedural fairness — or there is not, in which case the questions do not serve the stated purpose of this inquiry.

→ **FOR THE HEALTH SERVICE: please confirm whether any performance or conduct process concerning me is on foot or contemplated, and if so provide particulars.**

→ **THEN, IF APPROPRIATE, FOR MY PRACTITIONER: "Does the employee have capacity to participate in ordinary workplace meetings and supervision, and are any supports required?"**

The premise of Question 2 is separately incorrect and is addressed at Part 5.

4.2 QUESTION 3 — is the workplace safe?

The question asks whether I am "medically fit to return to their substantive role under the existing reporting arrangements, including working with and reporting to their current line manager." That asks a treating doctor to certify that a workplace arrangement is safe. Under section 19 of the Work Health and Safety Act 2011 that duty rests with the

Health Service; under the Managing the risk of psychosocial hazards at work Code of Practice 2022 a psychosocial hazard is a feature of the work, identified and controlled by the PCBU; and G03 clause 1 requires proper consideration of that Code. A doctor can advise on me; a doctor cannot assess the Health Service's reporting arrangements.

The letter of 7 July 2026 records that the Health Service "does not have sufficient information to understand the nature of these matters or assess whether they present any psychosocial hazards or risks". That gap is real, and it is closed by the Health Service consulting and assessing — sections 47 to 49 require consultation with workers directly affected, and none has occurred since 26 June 2026. For completeness, the matters concern how work in the unit is organised and decided: how patient safety concerns raised at the switchboard are escalated; how changes to work and roster have been introduced; fatigue and recovery between shifts; and staff input into decisions affecting the work. They are workplace matters, which is why the assessment belongs with the Health Service.

→ **FOR THE HEALTH SERVICE: a risk assessment of the Switchboard work environment addressing the psychosocial hazards identified. Clause 7.2.1 of the certified agreement provides the mechanism.**

→ **THEN FOR MY PRACTITIONER: "Are any adjustments recommended in relation to supervision arrangements, and if so, what are they in functional terms?"**

4.3 QUESTION 6 — what are the genuine occupational requirements?

The question asks whether I can "fulfill the full inherent requirements of [my] Role ... without restrictions or modifications to duties."

This applies the wrong test. G03 states its purpose as assisting employees "to meet the genuine occupational requirements of their role by applying principles of reasonable adjustment," and at clause 3 provides that adjustments "should support the employee to undertake the genuine occupational requirements of their role, without risk to their own health and safety or that of others in the workplace." Section 34 of the Anti-Discrimination Act 1991 is to the same effect. Neither asks whether an employee can perform a role without adjustment.

A clinician also cannot determine which duties are genuine occupational requirements. That is the employer's determination. The Health Service has itself already made part of it: its letter states that "established process for client complaints received by Switchboard employees is for immediate escalation and management by the Health Service Client Liaison Officer and/or Manager, Switchboard Services," and the Role Description at Attachment 2 contains no complaint-handling responsibility.

→ **FOR THE HEALTH SERVICE: identify which duties of the role it says are genuine occupational requirements, having regard to the Role Description at Attachment 2.**

→ **THEN FOR MY PRACTITIONER: "Can the employee perform the genuine occupational requirements of the role with the adjustments you have recommended?"**

4.4 QUESTION 9 — can the adjustments be accommodated?

The question asks: "If we are not able to accommodate the restrictions you have recommended, is Mr Shepherd able to safely return to the workplace?"

This presupposes an inability to accommodate before any accommodation has been assessed. G03 clause 2 provides that unjustifiable hardship "is tested against the whole organisation, not a division or unit within the organisation," and that "the onus is on Queensland Health, as the employer, to prove an adjustment is unreasonable, not on the person to prove that it is reasonable."

The question therefore asks my doctor to answer the final step first, on a hypothetical the Health Service has constructed and not yet tested, and on which the onus is not mine.

→ **FOR THE HEALTH SERVICE: assess the recommended adjustments against the whole organisation. If it considers any cannot be accommodated, identify which and its reasons.**

→ **THEN, IF NECESSARY, the question can be put to my practitioner properly.**

PART 5 — THE QUESTION I ASK BE WITHDRAWN, AND A CORRECTION OF FACT

5.1 WITHDRAWAL REQUESTED

Q1(b) "What is the clinical basis for identifying these specific workplace stressors as contributing to the exacerbation of Mr Shepherd's condition." This is not a question a general practitioner can properly answer.

The clinical basis for a psychiatric causal formulation is a matter for a psychiatrist. It is also a question I ask be

withdrawn. For the purpose of identifying controls, what matters is that the stressors exacerbate my condition and what controls address them — both of which are obtained by Questions 1(d) and 8. The clinical reasoning as to causation goes to a matter in issue in WC/2024/227 and is not necessary for the stated purpose of this inquiry.

I record that section 124(1) of the Anti-Discrimination Act 1991 provides that a person "must not ask another person, either orally or in writing, to supply information on which unlawful discrimination might be based," and that under section 124(3) the defence requires the respondent to prove "that the information was reasonably required for a purpose that did not involve discrimination." Impairment is a protected attribute. I raise this not as a complaint but so that the Health Service can satisfy itself that each question it presses meets that standard.

5.2 CORRECTION OF FACT

Question 2 is premised on the statement that "the Health Service is not aware of any concerns being raised for appropriate management."

That statement is incorrect. Concerns have been raised in writing on multiple occasions, including a public interest disclosure (PID 24-ESU-1130); a written patient safety report; correspondence to Human Resources over a period exceeding two years concerning fatigue, rostering and the management of directory information; a written complaint to the A/Director, Human Resources Business Partnering dated 3 October 2025; and the matters in WC/2024/227, in which the Health Service filed an objection dated 5 June 2026 under the signature of the Health Service Chief Executive.

I do not ask the Health Service to agree with any of those concerns. I ask only that my treating practitioner not be asked to answer a question which assumes they were never made. I request that the correction be provided to Dr Ma in writing before he is asked to answer any question resting on that premise.

PART 6 — DELEGATION AND CONFLICT OF INTEREST

This is not a personal complaint about any individual. I make no allegation and I ask for no finding about anyone. My concern is with how the process is being run — who is deciding, on what material, and under what instrument — and with the support and adjustment that ordinarily apply when an employee is certified fit with restrictions. G03 clause 1.1 provides for temporary adjustments so an employee may "continue work during their recovery"; the Workplace rehabilitation standard requires suitable duties to be actively sought and the worker's connection with the workplace maintained, for "either a work or non-work-related injury/illness". I obtained the checklist the Health Service required for that purpose. It certified me fit with restrictions. What followed was exclusion rather than adjustment.

Due process is the other half: being told what power is being used and who decides; a fair opportunity to respond before a serious employment consequence; consultation on the matters in issue; deadlines that are not one-sided; and decisions made by an officer independent of the matters they concern — what the Managing the risk of psychosocial hazards at work Code of Practice 2022 calls organisational justice. I do not say any tribunal has found a denial of fair process; I say that process has not been applied, and I ask that it be.

I asked on 28 July 2026 who is making decisions about me and whether any conflict of interest has been identified. I have not received a response. Both letters of 31 July 2026 are written in the first person of the decision-maker and signed by Mr Hughes, Director, Corporate Services; question 3 asks my doctor about the reporting arrangements within that same directorate, and the Manager who gave the advice of 2 July 2026 reports within it. The officer determining my return therefore sits within the arrangements in issue. I ask the Health Service to confirm: (a) the source and instrument of the delegation; (b) whether any conflict has been identified, declared and managed, and how; and (c) whether the decisions of 2 and 3 July 2026 were made with the knowledge or approval of the officer now determining my return.

6.1 A PROPOSED MANAGEMENT OF THE CONFLICT

I propose the ordinary separation of functions: the clinical material held exclusively by the Injury Management team on a medical-in-confidence basis — which is what the Checklist itself recommends at Additional Comments 4(c); any decision concerning my capacity or return made by an officer outside Corporate Services; and implementation — rostering, and putting the adjustments in place — retained within Corporate Services. The officer steps out of the decision, not out of the workplace. I record, because it bears on the assessment, that on 3 October 2025 I made a formal written request for review to the A/Director, HR Business Partnering concerning a process initiated by the officer who has signed the letters of 31 July 2026; I do not reopen it here. Code of Conduct clause 1.2 provides that a conflict is not wrongdoing in itself — failing to disclose and manage one is likely to be.

I have written separately to the Chief Executive today about the conflict and the handling of my personal information, because section 89 of the Public Sector Act 2022 makes the chief executive the person to whom a conflict is disclosed. That letter deals with the matter in full.

PART 7 — TIME

7.1 DEADLINES IN THIS MATTER HAVE RUN IN ONE DIRECTION

I met the deadline of 2:00pm on 3 July 2026; the checklist was provided at 2:32pm and certified me fit with restrictions. At 3:18pm I asked whether I should attend. Five weeks later that has not been answered, no instrument authorising my exclusion has been identified although I have asked three times, no date has been given by which I might return or any decision would be made, and on 30 July 2026 I was told the matter was "progressing for delegate approval" and that the Health Service was "unable to guarantee" my return.

On 31 July 2026 the Director, Corporate Services set me seven calendar days, with a consequence: a decision regarding my ability to perform my role on the available information. The letter does not say what that decision is. I ask that it be identified — what decision, and under what instrument. If it is a decision that I return to work, I welcome it and no consequence arises. If it is a decision of another kind, I am entitled to know now, and to the procedural fairness that attaches to it. This is the second occasion an adverse position has been foreshadowed without an instrument: on 15 July my return was said to "remain at the discretion of the Employer".

I do not object to a deadline. I note only that I have met every deadline set for me, and in five weeks without income I have been given none in return. I ask for the extension below, and for a date by which the Health Service will tell me whether I am returning to work.

7.2 THE EXTENSION

The letter allows seven days, expiring 7 August 2026, and expressly contemplates a reasonable extension. I request one. The letter arrived at 11:43am on 31 July, two hours before a consultation with Dr Ma of which the Health Service had notice; because the question set is in issue I cancelled it rather than waste a cost, and will rebook once the set is settled. I propose two parts: (a) Dr Ma's questions — 21 days from the Health Service confirming the final question set; (b) specialist questions — on a timeframe set once it confirms which it presses and that it will meet the cost. I do not seek delay; the exclusion is costing me my income, and it cannot be reasonable to maintain an unpaid exclusion for a period created by the Health Service's own requirements.

PART 8 — DISCLOSURE AND COSTS

I adopt option 1 and will obtain the information from my practitioners and provide it. I do not provide an authority under option 2. G03 clause 3 acknowledges "the right of the person... to choose how disclosure of information occurs", and that information need only be shared where genuine occupational requirements or safety are at risk and reasonable adjustment is required. My election exercises a right the policy expressly recognises.

As to handling, I ask that any medical information I provide be: (a) held by the Injury Management Consultant on a medical-in-confidence basis; (b) not provided to my line manager or Switchboard Services — a line manager requires the adjustments, not the clinical information behind them; and (c) used only for the stated purpose. Please confirm that, and identify any person outside the Health Service to whom it is proposed to be provided. My separate letter to the Chief Executive of today's date deals with information handling in full.

As to costs, the letter of 31 July 2026 confirms that MSH will meet the reasonable costs of the reports and directs invoices to you. I have proceeded on that basis; both practitioners will invoice you directly, quoting MSH-INJ-5795. The consultation of 3 July 2026 (\$150.00, invoice 574370, paid 10 July 2026) is an earlier cost whose reimbursement I sought on 30 July 2026 and which remains outstanding.

PART 9 — RESPONSE REQUESTED

I ask for a written response confirming:

1. whether I am returned to my roster, and if not, the instrument relied on to maintain my exclusion and whether the period will be treated as paid special leave with my leave re-credited;
2. the delegate, the source of the delegation, how any conflict of interest has been identified and managed, and whether the separation of functions proposed at Part 6.1 is agreed — the medical information held exclusively by Injury Management, the decision made outside Corporate Services, and implementation retained within it;

3. whether any performance or conduct process concerning me is on foot or contemplated;
4. which duties of my role the Health Service says are genuine occupational requirements;
5. whether the Health Service will conduct the workplace risk assessment;
6. which questions it presses and which it withdraws;
7. that the information will be used only for the stated purpose, who outside the Health Service it is proposed to be provided to, and that it will be handled within the Health Service as set out at Part 8; and
8. what decision is meant by "a decision regarding your ability to perform your role", and the instrument under which it would be made;
9. a date by which the Health Service will tell me whether I am returning to work, as sought at Part 7.1; and
10. that my substantive position continues to be held for me, as set out in the accompanying Proposal for Return to Work; and
11. its response to the accompanying Proposal for Return to Work, which is made as a request under clause 10.3.2 of the certified agreement and to which clause 10.3.6 applies.

I confirm again that I am engaging with this process and want to return to work.

Cory Lea Shepherd AO3, Switchboard Services, Logan Hospital 0417 400 227 · coryshepherd1@hotmail.com

Proposal for Return to Work

Accompanying the Response to the Request for Medical Information · MSH-INJ-5795

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital, Metro South Health

To: Human Resources, Logan Bayside Health Network

LBH.HRTeam1@health.qld.gov.au

Date: 3 August 2026

1. WHAT THIS DOCUMENT IS

This document sets out my proposal for returning to work. It accompanies my response of today's date to the Request for Medical Information, and is presented separately so that it can be considered — and, internally, routed — on its own. The confirmations I seek are listed at Part 9 of the response. The proposal at section 3 below is also the subject of my separate request of today's date under clause 10.3.2 of the certified agreement.

2. THE PROPOSAL

I have been held out of the workplace since 3 July 2026 without wages, with my accrued leave debited, and self-funding the medical costs of this process. G03 clause 1.1 contemplates temporary adjustments which allow an employee to "continue work during their recovery," and clause 1.1.1 confirms that workplace rehabilitation support is available to employees "who have an injury/illness impacting their current ability to perform their role, with no claim or entitlement to compensation (health management case)." Clause 1 provides that "Managers are responsible for ensuring work environments and work, do not unreasonably prevent employment for people."

The Health Service's Workplace rehabilitation standard (QH-IMP-401-5, 7 January 2026) is also engaged. It provides that the line manager or delegate shall "identify and actively seek ... meaningful suitable duties, and ensure suitable duties are made available wherever reasonably practicable"; that the line manager "is best placed to be the primary contact for the worker wherever possible and facilitate the injured workers on-going connection with the workplace"; that "regular, transparent and timely communication" be maintained with the worker "whether at work or off work"; and that the worker be informed of "the appropriate dispute resolution process to follow in such cases where the worker does not agree with elements of a proposed workplace rehabilitation plan or suitable duties program". The standard applies to a worker requiring rehabilitation "for either a work or non-work-related injury/illness".

I have been offered no suitable duties — interim duties were declined in any capacity. I have been held away from the workplace rather than connected to it. I have not been informed of any dispute resolution process. I ask that each of those be addressed.

I also record that I was not consulted before the decision of 3 July 2026, or since. Section 47 of the Work Health and Safety Act 2011 requires consultation with workers who are "directly affected by a matter relating to work health or safety". Section 48 requires that I be given a reasonable opportunity "to contribute to the decision-making process relating to the matter". Section 49 requires consultation when identifying hazards and assessing risks, when deciding how to eliminate or minimise them, when proposing changes that may affect the health or safety of workers, and when making decisions about the procedures for monitoring the health of workers. The present request is a procedure for monitoring my health.

I propose the following. Each item is within the Health Service's existing processes and, so far as I can see, requires no material expenditure and no restructuring.

- (a) RETURN TO DUTY. That I return to my rostered shifts now, on the restrictions certified in the Employee Capabilities Checklist of 3 July 2026, while the balance of this inquiry is completed. If the exclusion is maintained, that the instrument relied on be identified, that the period be treated as paid special leave, and that the leave debited since 3 July 2026 be reversed.
- (b) COMPLAINT HANDLING. That the Health Service confirm in writing that complaints received by Switchboard staff are escalated in accordance with the process it has itself described. This requires no change to anything.
- (c) ROSTER PREDICTABILITY. That my roster be published in accordance with the certified agreement and not altered without my agreement other than in an emergency.

- (d) ORDINARY PROCEDURAL FAIRNESS. That any concern about my performance or conduct be raised with me in writing, with particulars, in accordance with the applicable directive, and that I be afforded a support person.
- (e) A SEPARATE CONTACT POINT. That leave, injury management and human resources matters concerning me be handled by an officer outside Switchboard Services rather than by my line manager, who wrote to me on 2 July 2026 concerning leave types and my return to duty. This does not alter day-to-day supervision and involves no change to reporting lines.
- (f) WORKPLACE RISK ASSESSMENT. As set out at Part 4.2.
- (g) REVIEW. That the arrangements be reviewed at six weeks, or earlier if either party considers it necessary.
- (h) GRADUATED RETURN. If the Health Service prefers, I am willing to return on a graduated basis by agreement. I am not seeking to avoid any part of my role.

If any part of this proposal cannot be accommodated, I ask that the Health Service identify which part and its reasons, having regard to G03 clause 2.

3. CONTROLS I WOULD BE WILLING TO SEE PUT IN PLACE

I record these subject to two qualifications I do not waive: identifying hazards and determining controls is a function of the Health Service under Part 3.1 of the Work Health and Safety Regulation 2011, not of the worker; and the controls that are appropriate are ultimately a matter for clinical opinion, which question 1(d) of the Health Service's own request seeks.

- (a) A telephone call one hour before each rostered shift, to confirm I am attending; if unanswered or attendance is not confirmed, a second call; if still not confirmed, the on-call operator is contacted and cover arranged. The measure is contingent, not standing: on the shifts I attend — most of them — it costs one telephone call, and where I am not attending, that is established an hour ahead rather than at start time. An on-call arrangement has existed in the unit since 15 April 2024; this applies it, and would have addressed a substantial part of the attendance history recorded in the correspondence of 8 September 2025. It is a confirmation of attendance, not an assessment of my condition — no colleague needs to know anything about my health for it to work.
- (b) A defined escalation pathway I may initiate during a shift if my condition is acutely exacerbated — who assumes the board, how the shift is closed out — so the response is planned rather than improvised.
- (c) Continuation of the existing complaint-handling arrangement recorded in the Health Service's own letter: immediate escalation to the Client Liaison Officer and/or Manager. This requires no change to anything.
- (d) Continuation of the rostered pattern that has applied over the preceding twelve months. The Checklist records it as "the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration" and a "continuation of existing arrangement". The Health Service set that roster; where my condition was exacerbated I managed it myself, from my own sick and annual leave — which is why, on 3 July 2026, my paid sick leave was exhausted. I am not asking for the pattern to be reduced. I am asking that it continue, with the variability managed by a process rather than by the exhaustion of my entitlements. If it is now said the role requires something different, I ask what changed, and when.
- (e) Injury management correspondence directed through the Injury Management team, consistent with the Checklist's own recommendation.

The only conduct matter ever raised with me is attendance and lateness; the measures at (a) and

(b) are directed at precisely that. If any measure cannot be accommodated I ask that the Health

Service identify which and why, noting G03 clause 2: unjustifiable hardship is "tested against the whole organisation, not a division or unit", and "the onus is on Queensland Health, as the employer, to prove an adjustment is unreasonable".

4. THE STATUS OF THIS PROPOSAL — A REQUEST UNDER CLAUSE 10.3

So that the Health Service is not left to determine for itself what kind of document this is, I identify the provision the proposal is made under.

Clause 10.3.2 of the certified agreement provides that, in accordance with the Industrial Relations Act 2016, an employee "may ask the employer for a change in the way the employee works, including the employee's ordinary hours of work." Section 27(1)(c) of that Act puts the matter beyond doubt for a case such as this one: the request may be for "a change to the way the employee works, for example, the use of different equipment as a result of a disability, illness

or injury."

The measures at sections 2 and 3 are a change in the way I work, sought as a result of an illness. I have made that request formally, by a separate letter of today's date headed "Request for a change in the way I work", so that there is no question about what it is or when it was received. That letter sets the change out in the form clause 10.3.3 requires and states the reasons for it. Sections 2 and 3 of this document are the fuller explanation of the same proposal.

I ask that the two be read together, and that the request be dealt with under clauses 10.3.4 to 10.3.6. Nothing in this Part is intended to displace a discussion by agreement, which I would much prefer, and I remain willing to consider any alternative the Health Service or my practitioners consider more appropriate.

For completeness, the request under clause 10.3 is separate from, and does not answer, the matters at Parts 4, 8, 9, 10 and 11 of this document, which are not made under that clause.

5. MY SUBSTANTIVE POSITION

The letters of 31 July 2026 describe my employment as "Administration Officer (AO), Switchboard Operations, Corporate Services, Logan Hospital (LH), Metro South Health (MSH) on a Permanent Full-time basis 76 hours per fortnight."

I ask the Health Service to confirm:

- (a) that the position so described continues to be held for me, and that no permanent appointment to it has been made or is proposed to be made while I am held out of the workplace;
- (b) whether the shifts I would ordinarily work are being performed by another employee. I ask because it bears directly on Question 9. The Health Service has asked my doctor to advise whether I can safely return if it is "not able to accommodate the restrictions". If the duties I would perform are being performed by someone else, the work is available, and the premise of that question requires examination before it is put to anyone; and
- (c) what, if anything, has been communicated within the workplace about my absence.

I raise (c) because my absence has been the subject of enquiry among colleagues. I would prefer that whatever is said about it is accurate.

I raise these matters only to have the position confirmed in writing.

I confirm again that I am engaging with this process and want to return to work.

Yours sincerely,

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital

0417 400 227 · coryshepherd1@hotmail.com

Arrangements for the medical appointments, and costs

Request for Medical Information dated 31 July 2026 · MSH-INJ-5795

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital, Metro South Health

To: Human Resources, Logan Bayside Health Network

LBH.HRTeam1@health.qld.gov.au

Date: 3 August 2026

1. THE APPOINTMENTS, AND WHAT PRECEDES THEM

The letter of 31 July 2026 asks that the information sought be returned within seven calendar days, and states that if it is not received a decision will be made regarding my ability to perform my role on the information then available.

I am ready to obtain that information, and one step precedes it — a step which sits with the Health Service. My response of today's date proposes an allocation of the nine questions and asks that one be withdrawn. Before I put questions to my practitioners and incur costs on the Health Service's account, I ask the Health Service to confirm which questions it presses and to whom each is directed.

On that confirmation I will rebook both consultations — with Dr Ma, and with my treating psychiatrist — within three business days, and I will advise the Health Service of both appointment dates the day they are made. The consultation with Dr Ma of 31 July 2026, of which the Health Service had notice, was cancelled only because the letter arrived two hours before it and put the question set in issue; I did not consider it reasonable to incur a cost, on the Health Service's account, on questions the Health Service may not press.

If I have not received confirmation within seven days of this notice, I will rebook both consultations in any event and put the questions as allocated in my response, and I will tell the Health Service what was asked of whom. The information will follow as soon as the reports are received.

2. THE TIMEFRAME

The seven-day period runs to 7 August 2026. Reports of this kind cannot ordinarily be obtained within seven days of being asked, and the Health Service will know that from its own experience: its letter of 7 July 2026 also set a seven-day period, and on 15 July 2026 it advised that it was itself unable to commit to a timeframe for the same information.

I ask that no decision be made regarding my ability to perform my role on the basis that information has not been provided, while the step which precedes it — the Health Service's confirmation of the question set — remains outstanding, and while the reports it makes necessary are then obtained. If the Health Service intends to make such a decision earlier, I ask to be told in advance, and to be told what decision is contemplated and under what instrument.

3. COSTS

I do not ask for anything under this heading. The position is already settled by the Health Service's own letter, and I record it only so that the arrangement is clear.

The letter of 31 July 2026 states:

"MSH will meet your reasonable costs of preparing this report and we appreciate your assistance with this request. Please send invoices to Michelle Harrison, Injury Management Consultant, Human Resources, Logan and Beaudesert Health Services via email at lbh_InjuryManagement@health.qld.gov.au."

I accept that arrangement and will proceed on it: when the consultations are rebooked, each practitioner will be instructed to render their account directly to Ms Harrison at that address, quoting MSH-INJ-5795, and not to me. I will advise the expected fees once confirmed.

For completeness, and because they are separate matters:

- (a) THE REPORTS NOW SOUGHT. The arrangement above covers them. The questions I propose to put to my treating psychiatrist are questions from the Health Service's own request, allocated as set out in my response, and I proceed on the basis that they fall within it. If the Health Service takes a different view as to any part, I ask to be told before the appointments so that no cost is incurred on a footing that is not agreed.

(b) THE CONSULTATION OF 3 JULY 2026. That is an earlier and separate cost. It was invoiced to me personally — My Doctors Clinic, invoice 574370, 3 July 2026, \$150.00 — and I paid it in full on 10 July 2026, receipt N471065713633. I asked on 30 July 2026 that it be reimbursed to me. That request has not yet been answered and I simply record that it remains outstanding.

I record, without making anything of it, that I have been without wages since 13 July 2026 and have to this point met the medical costs of this process from my own funds.

4. WHAT THIS NOTICE DOES NOT DO

(a) This notice does not vary my response of the same date, and does not withdraw the request at Part 5 of that response that question 1(b) not be put.

(b) Nothing in it is a concession that I am unfit to attend work, or that my attendance properly awaits the reports. I hold a certificate, and the Health Service's letter of 7 July 2026 records that I have been "certified fit to return to work effective from 3 July 2026". My position on attendance and pay is dealt with separately.

(c) Nothing in it is a request for leave of any kind.

I confirm again that I am engaging with this process and want to return to work.

Yours sincerely,

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital

0417 400 227 · coryshepherd1@hotmail.com

Request for a change in the way I work

Clause 10.3.2, Certified Agreement (No. 12) 2025 · s 27, Industrial Relations Act 2016 (Qld)

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital, Metro South Health

Employee reference MSH-INJ-5795

To: Human Resources, Logan Bayside Health Network

LBH.HRTeam1@health.qld.gov.au

Date: 3 August 2026

1. WHAT THIS DOCUMENT IS

This is a request under clause 10.3.2 of the certified agreement for a change in the way I work.

I make it as a separate document, rather than within my response to the Request for Medical Information of 31 July 2026, so that there is no question about what it is or when it was received.

Clause 10.3.2 provides that, in accordance with the Industrial Relations Act 2016, an employee "may ask the employer for a change in the way the employee works, including the employee's ordinary hours of work". Section 27(1)(c) of that Act provides that the request may be for "a change to the way the employee works, for example, the use of different equipment as a result of a disability, illness or injury".

Clause 10.3.3 requires that the request be in writing, state the change in sufficient detail to allow a decision to be made, and state the reasons. Parts 2 and 3 below do each of those things.

I make the request under two separate limbs of section 27(1), and I ask that they be considered separately.

(a) **ORDINARY HOURS** — Part 2.1. This is made under section 27(1)(a), which concerns "the employee's ordinary hours of work", and under clause 10.3.2, which is in the same terms. That limb stands on its own. It does not depend on illness or injury, and neither section 27, nor section 28, nor clause 10.3 makes medical information a precondition to making such a request or to deciding one. My reasons for it are matters of rostering history and are set out at Part 3.

(b) **THE WAY I WORK** — Parts 2.2 to 2.5. These are made under section 27(1)(c), which concerns "a change to the way the employee works, for example, the use of different equipment as a result of a disability, illness or injury".

I separate them because a decision on one is not a decision on the other, and because the question of my ordinary hours does not require, and should not await, information sought for a different purpose. If the Health Service is not in a position to decide (b) at present, I ask that it decide (a), and say so.

2. THE CHANGE I AM ASKING FOR

2.1 MY ORDINARY HOURS

That my rostered pattern continue at the level at which it has in fact operated over the twelve months preceding the Employee Capabilities Checklist of 3 July 2026 — approximately 0.6 of the shifts of my full-time permanent line of 76 hours per fortnight, worked from within that line, being the average I have actually worked across those twelve months, with the precise figure to be established from the Health Service's own roster and payroll records as set out below. My appointment to that line is not varied by this request.

The Employee Capabilities Checklist describes that level, at Additional Comments 3, as a "reduced pattern of approximately six shifts per fortnight", and records that it "is the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration, and maintaining rather than increasing it best supports recovery while minimising risk of exacerbation". The first page records the arrangement as a "continuation of existing arrangement".

I am not asking for any reduction from the pattern I have been working. I am asking that the existing pattern continue. It was set by the Health Service, worked by me, and sustained across twelve months. No concern was raised with me at any time that it was insufficient, and no request was made that I work more.

I record for completeness that my substantive appointment is full-time at 76 hours per fortnight, and that the pattern described above is less than that. It is, however, the pattern the Health Service in fact rostered and accepted throughout the twelve months in question, and the Employee Capabilities Checklist describes it as a "continuation of existing arrangement". Clause 10.3.2 expressly contemplates a request concerning "the employee's ordinary hours of work",

which is why I have made the request under that clause rather than assuming the position.

If the Health Service takes the view that the requirements of the role cannot be met on that pattern, I ask that it identify what changed, and when, given that the pattern operated for twelve months without any concern being raised with me.

The Health Service holds the roster and payroll records from which the precise figure can be established. I ask that it be established from those records, and that the figure so established be the one used.

I also record that a temporary reduction of hours was raised with me by the Health Service itself, as a form of support, in the course of a discussion recorded in writing on 30 June 2025. I did not take it up at the time. I mention it only to show that an adjustment of this kind has been contemplated within Switchboard Services before, and is not a novel proposal.

2.2 THE WAY I WORK — ATTENDANCE CONFIRMATION

- (a) A telephone call one hour before each rostered shift, to confirm that I am attending.
- (b) If that call is not answered, or attendance is not confirmed, a second call.
- (c) If attendance is still not confirmed after the second call, the on-call operator is contacted and cover is arranged for the shift.

The measure is contingent, not standing. The on-call is engaged only if the two calls do not produce a confirmation, so it carries no cost on any shift I attend. On the shifts I do attend, which is most of them, the arrangement costs one telephone call.

An on-call arrangement already exists in the unit. On 15 April 2024 the team was advised that after-hours on call had been added, with the days highlighted on the roster and a contact process for urgent matters. What I am asking for is the application of a mechanism Switchboard Services already operates, not the creation of a new one.

To be clear about what this is and is not: it is a confirmation of attendance, of the kind any roster requires, made early enough to be useful. It is not an assessment of my condition, and I am not asking that colleagues on shift observe, assess or report on my mental state. That would place a burden on staff who are not qualified to carry it, would require my health information to be known to co-workers, and would itself be a workplace stressor. No colleague needs to know anything about my health for (a) to (c) to work. They need only know whether the shift is confirmed.

2.3 THE WAY I WORK — IN-SHIFT ESCALATION

A defined escalation pathway which I may initiate during a shift if my condition is acutely exacerbated — including who assumes the board and how the shift is closed out — so that the response is planned rather than improvised.

2.4 THE WAY I WORK — COMPLAINT HANDLING

Continuation of the existing arrangement recorded in the Health Service's own correspondence: immediate escalation of complaints received by Switchboard staff to the Client Liaison Officer and/or the Manager, Switchboard Services. This is the Health Service's established process and it requires no change to anything.

2.5 THE WAY I WORK — CORRESPONDENCE ROUTING

That injury management correspondence concerning me be directed through the Injury Management team, Human Resources. This is what the Employee Capabilities Checklist itself recommends at Additional Comments 4(c): "return-to-work coordination and correspondence through the Injury Management team, Human Resources."

2.6 REVIEW

That the arrangement be reviewed at six weeks, or earlier if either party considers it necessary.

I am willing to discuss any of the above, and any alternative the Health Service or my treating practitioners consider more appropriate. If the Health Service would prefer a graduated commencement, I am willing to return on a graduated basis by agreement.

3. THE REASONS FOR THE CHANGE

3.1 The pattern was sustained out of my own leave, and that is what ran out. The roster was

designed by the Health Service. Where my condition was exacerbated I managed it myself, by sick or annual leave; no adjustment was sought and none was provided. That is why, on 3 July 2026, my paid sick leave was exhausted — in the course of managing, without assistance, a condition arising in the workplace.

3.2 The measures replace that with a process. They do not change the roster; they replace an

arrangement under which exacerbations are absorbed by my leave balances with one under which they are managed. Distress rises as a rostered shift approaches, and there have been occasions where I have called in at short notice; a late call leaves the board short and the preceding shift unable to leave. The sequence at 2.2 establishes non-attendance an hour ahead instead.

3.3 It is directed at the only matter ever raised with me — attendance and lateness. No concern

about the quality of my work has ever been put to me, and the correspondence of 31 July 2026 records that the Health Service is not aware of any performance or conduct matters raised for management. Had these measures been in place, they would have addressed a substantial part of the attendance history in the correspondence of 8 September 2025.

3.4 The applicable policy contemplates exactly this. G03 clause 1.1 provides temporary

adjustments so an employee may "continue work during their recovery", and clause 1 makes managers responsible for ensuring work environments "do not unreasonably prevent employment for people". The adjustment is the thing that allows work to continue while recovery occurs.

4. THE DECISION, AND WHO SHOULD MAKE IT

Clause 10.3.4 provides that the Health Service may grant this request, grant it in part or subject to conditions, or refuse it. Clause 10.3.5 provides that it may grant it in part or subject to conditions, or refuse it, "only on reasonable grounds".

I ask that the decision be made by an officer outside Corporate Services.

I do not ask that Corporate Services be removed from the management of my employment, and I make no allegation against any individual. Part of what this request concerns is the reporting arrangements within Corporate Services, and the officers who would otherwise decide it sit within those arrangements. Clause 1.2 of the Code of Conduct for the Queensland Public Service provides that a conflict of interest is not wrongdoing in itself, but that failing to disclose and manage one is likely to be wrongdoing. Section 89 of the Public Sector Act 2022 is to the same effect.

The separation I propose is the ordinary one: the clinical material held by Injury Management on a medical-in-confidence basis; the decision made outside Corporate Services; implementation — rostering, and putting in place whatever is decided — retained within Corporate Services. The officer steps out of the decision, not out of the workplace. This requires no restructuring and no finding to be made about anyone.

I ask that the officer who makes the decision be identified to me, together with the source of the delegation relied on.

5. THE FRAMEWORK THE DECISION SITS IN

I set this out for one reason only. Clause 10.3.5 permits the Health Service to grant the request in part or subject to conditions, or refuse it, "only on reasonable grounds". Grounds are reasonable against the whole of the law that applies, not against the agreement alone. If the request is refused in whole or in part, I would like to be able to understand the reasons given under clause 10.3.6 by reference to the same framework the Health Service is applying.

5.1 THE OBLIGATION

The Anti-Discrimination Act 1991 provides at section 15(1) that a person must not discriminate "in any variation of the terms of work", or "by treating a worker unfavourably in any way in connection with work". Section 15(2) provides that "dismissing includes ending the particular work of a person by forced retirement, failure to provide work or otherwise".

Section 11 provides that indirect discrimination happens if a person imposes a term with which a person with an attribute does not or is not able to comply, and which is not reasonable. Impairment is a protected attribute under section 7.

5.2 THE EXEMPTIONS, AND WHAT THEY REQUIRE

Sections 35 and 36 provide that it is not unlawful to discriminate on the basis of impairment where the person would require special services or facilities and supplying them would impose unjustifiable hardship, or where the circumstances of the impairment would impose unjustifiable hardship.

Those provisions are exemptions, and each of them turns on unjustifiable hardship. Section 5 provides that whether something would impose unjustifiable hardship depends on all the relevant circumstances of the case, including the nature of the services or facilities, their cost and the number of people who would benefit or be disadvantaged, the

financial circumstances of the person, the disruption that supplying them might cause, and the nature of any benefit or detriment to all people concerned.

HR Policy G03 puts the same test in the Health Service's own words, at clause 2: unjustifiable hardship is "tested against the whole organisation, not a division or unit within the organisation", and "the onus is on Queensland Health, as the employer, to prove an adjustment is unreasonable, not on the person to prove that it is reasonable".

I would add that the Health Service is a public entity for the purposes of the Human Rights Act 2019, and section 58 of that Act requires a public entity to act and make decisions compatibly with human rights and to give proper consideration to relevant human rights in doing so.

5.3 WHY I RAISE IT, AND WHAT I AM NOT DOING

I have made no complaint under the Anti-Discrimination Act, and this letter is not one. I make no allegation against anyone and I ask for no finding to be made about anyone.

I raise it because the measures at Part 2 are modest ones, and I would like the reasons for any refusal to engage with that. On the shifts I attend, the arrangement at 2.2 costs one telephone call. The roster at 2.1 is the roster the Health Service already sets. The complaint-handling arrangement at 2.4 is the Health Service's own established process. The correspondence routing at 2.5 is what the Employee Capabilities Checklist itself recommends.

If the Health Service considers that any of the measures would impose unjustifiable hardship, I ask only that it identify which measure, and what the hardship is. That is the same thing clause 10.3.6 requires, expressed in the language of the Act.

6. THE TIME FOR THE DECISION

Clause 10.3.6 provides:

"The employer must give the employee written notice about its decision within 21 days after receiving the request. If the employer decides to grant the request in part or subject to conditions or to refuse the request, the written notice about the decision must state the reasons for the decision, outlining the reasonable grounds for granting the request in part or subject to conditions or for the refusal."

Section 28(3) of the Industrial Relations Act 2016 is in the same terms. Section 28(4) provides that a notice granting the request in part or subject to conditions, or refusing it, must state the written reasons outlining the reasonable grounds, and must state that the Commission has jurisdiction to hear and decide a dispute over the request under chapter 6. Section 29 provides that if the written notice is not given within 21 days after the request is received, the employer is taken to have decided to refuse the request.

Twenty-one days from the date of this letter is Monday 24 August 2026.

I record these provisions because a date has not been available in this matter at any stage. I have asked several times to be told when I would know whether I am returning to work, and I have not been given a date. The certified agreement provides one, and I would much prefer that the request be dealt with by agreement well before it.

7. WHAT THIS REQUEST DOES NOT DO

For completeness:

- (a) This request is separate from my response of the same date to the Request for Medical Information. The matters raised there are not made under clause 10.3 and are not answered by a decision under it.
- (b) This request is separate from any dispute notified under clause 1.11 of the certified agreement.
- (c) Nothing in this request is a concession that I am unfit to attend work. I remain willing and available to work, and have been since 3 July 2026. Identifying hazards and determining control measures is a function of the Health Service under Part 3.1 of the Work Health and Safety Regulation 2011, and nothing here relieves the Health Service of that function or is offered as a substitute for the workplace assessment I have asked for.
- (d) I make this request without waiver of any entitlement, and it is not to be treated as a request for leave of any kind.
- (e) For the avoidance of doubt, the request at Part 2.1 concerning my ordinary hours is not made conditional on the provision of any medical information, and I ask that it not be decided on that footing. If the Health Service considers it cannot decide Part 2.1 without medical information, I ask that it identify the provision that makes such information a precondition to a decision under clause 10.3.4.

I confirm again that I am engaging with this process and want to return to work.

Yours sincerely,

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital

0417 400 227 · coryshepherd1@hotmail.com

Notice of dispute — Stage 1

Clause 1.11.2(a), Certified Agreement (No. 12) 2025

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital, Metro South Health

To: Ms Chloe Taylor, Manager, Switchboard Services

(immediate supervisor, for the purposes of clause 1.11.2(a))

Human Resources, Logan Bayside Health Network — LBH.HRTeam1@health.qld.gov.au

Cc: Ms Emily Petering, Industrial Officer, Together Queensland

Mr Heath Moran, Together Queensland

Date: 3 August 2026

Subject: Notice of dispute under clause 1.11 — application of the certified agreement to my attendance, roster, leave and pay from 26 June 2026

1. WHAT THIS NOTICE IS

This notice informs you, as my immediate supervisor, of the existence of a dispute, in accordance with clause 1.11.2(a) of the certified agreement.

Clause 1.11.2 provides that where there is "any disagreement between the parties as to the interpretation, application or implementation of this Agreement", the procedure in that clause applies. Clause 1.11.1 records that the objectives are "the avoidance and resolution of any disputes ... by measures based on the provision of information and explanation, consultation, co-operation and negotiation", and that "the emphasis will be on finding a resolution at the earliest possible stage in the process".

That is what I am seeking. I would prefer this resolved at Stage 1 and I am available to discuss it at any time.

2. THE SUBJECT MATTER OF THE DISPUTE

The dispute concerns the application and implementation of the certified agreement to my employment from 26 June 2026, in the following respects.

2.1 ATTENDANCE AND ROSTERED DUTY

I have not been permitted to attend my rostered shifts since 3 July 2026. On 2 July 2026 I was advised that the Health Service was "unable to facilitate" my return until a completed Employee Capacity Certificate was received, and asked to "wait for my advice to when your next returning shift will be".

The Employee Capability Checklist was returned to the Injury Management team on 3 July 2026 at 2:30pm and, a section having been identified as missing, resubmitted at 3:06pm that afternoon. The letter of 7 July 2026 records both times. The same letter records that "you have been certified fit to return to work effective from 3 July 2026". I asked the same afternoon to be told whether I should attend. I have not received an answer to that question.

The condition on which the advice of 2 July 2026 was expressed has therefore been satisfied. I have asked which instrument is relied on to maintain the position since, and have not received an answer.

2.2 LEAVE

Since 3 July 2026 my accrued leave has been debited, and from 13 July 2026 I have been recorded as absent without pay, in respect of a period in which I have been willing and available to work and have said so in writing. I dispute the application of the leave provisions of the certified agreement to that period.

2.3 ROSTER

I dispute the position, so far as it is taken, that my rostered pattern over the preceding twelve months is inconsistent with the requirements of my role. That pattern was set by the Health Service and worked without any concern being raised with me.

2.4 WORK HEALTH AND SAFETY MATTERS RAISED

Clause 7.1.11 provides that "the parties commit to ensure that appropriate feedback is provided to employees who raise workplace health and safety matters". Clause 7.1.5 records the parties' agreement to address, among other issues, "management of ill or injured employees", "psychosocial issues and implementation of Managing the risk of psychosocial hazards at work Code of Practice 2022", and "injured workers to have the opportunity to be re-trained in

alternative areas/departments". I have raised matters within each of those descriptions and have not received feedback on them.

2.5 PSYCHOSOCIAL RISK ASSESSMENT

Clause 7.2 provides for psychosocial risk assessments conducted using the resources in the Managing the risk of psychosocial hazards at work Code of Practice 2022, "in consultation with workers and Health and Safety Representatives". I have asked for a workplace risk assessment and none has been provided.

I record that the Health Service identified this itself. The letter of 7 July 2026 states:

"it is noted that your treating medical practitioner has advised that there are unresolved workplace matters involving line management. At this stage, the Health Service does not have sufficient information to understand the nature of these matters or assess whether they present any psychosocial hazards or risks that may affect your proposed return to work."

That was written on 7 July 2026, with further information said to be required "to enable the Health Service to assess any identified risks". So far as I am aware, no assessment has been undertaken in the four weeks since. I ask that one be conducted, and that I be consulted in it, as clause 7.2.2 contemplates.

2.6 THE PROCESS ITSELF

Since 26 June 2026 I have been asked to provide medical information about myself, held out of my rostered shifts, had my accrued leave debited and then been recorded as absent without pay, and told that a decision will be made regarding my ability to perform my role.

The letter of 15 July 2026 refers to employees "being managed through Health Management processes". That is the only description of this process I have been given.

I have a phrase. I do not have a process.

I ask to be told, in relation to it:

- (a) the instrument under which it is conducted — the Act, directive, clause of the certified agreement, or policy;
- (b) what its steps are, and where they are published;
- (c) who the decision-maker is; and
- (d) what decision it ends in.

I am not raising this as a complaint about anyone. I raise it because I cannot follow a process properly, or exercise whatever rights attach to it, without knowing those four things. If the answer to any of them is that there is none, that is also something I need to know.

The Workplace rehabilitation standard (QH-IMP-401-5) requires that a worker be informed of "the appropriate dispute resolution process to follow"; I have not been informed of any. Clause 1.11.1 makes "the provision of information and explanation" an objective of this procedure — this is a request for exactly that, and it is the first one I make.

3. THE STATUS QUO WHILE THE PROCEDURE IS FOLLOWED

Clause 1.11.4 provides:

"Subject to legislation, while the dispute procedure is being followed, normal work is to continue except in the case of a genuine safety issue. The status quo existing before the emergence of a dispute is to continue whilst the procedure is being followed. No party shall be prejudiced as to the final settlement by the continuation of work."

The status quo before the emergence of this dispute was that I was rostered and working. I ask that I be returned to my rostered shifts while the procedure is followed, in accordance with that clause. I am not asking for anything to be decided in my favour by doing so; clause 1.11.4 expressly provides that no party is prejudiced as to the final settlement by the continuation of work.

I anticipate the Health Service may say that the exception for "a genuine safety issue" applies. If that position is taken, I ask that it be put in writing and that the following be identified, so that it can be discussed at Stage 1:

- (a) the hazard identified, and the assessment on which it was identified;
- (b) the control measures considered, in the order required by section 36 of the Work Health and Safety Regulation 2011, and why each was rejected; and

(c) why the restrictions certified on the Employee Capabilities Checklist of 3 July 2026 do not address it.

I record that I hold a current certificate addressing my capacity, that I have proposed controls in writing, and that I have asked for the workplace assessment which would resolve the question.

4. WHAT I ASK AT STAGE 1

(a) That I be told, in relation to the "Health Management processes" referred to in the letter of 15 July 2026, the four matters at 2.6(a) to (d) — the instrument, the steps and where they are published, the decision-maker, and the decision it ends in.

(b) That I be told whether I am suspended from duty. If I am, I ask to be told by whom, under what provision, from what date to what date, and whether I am entitled to remuneration for the period. If I am not suspended, I ask to be told what my status is.

(c) That I be returned to my rostered shifts while this procedure is followed, in accordance with clause 1.11.4; or, if that is not agreed, that the instrument relied on to maintain the position be identified in writing.

(d) That the period from 3 July 2026 be treated as paid, and the leave debited since that date re-credited; and that, from the date of this notice, the rostered shifts of my permanent line be paid in full as they fall, until either I am returned to work, or the Health Service identifies in writing the power under which it declines my services without pay. If the basis given is the outstanding medical information, I ask that payment continue while that information is obtained within the reasonable period my response of today's date sets out — the information is being obtained, and the time it takes is not of my making. This is the pay dimension of the status quo under clause 1.11.4: the position before this dispute emerged was that I was rostered, working, and paid.

I put the basis for that shortly. Throughout the period I have been ready, willing and able to work. I have said so in writing and I have not withdrawn it. I hold a certificate, and the Health Service's own letter of 7 July 2026 records that "you have been certified fit to return to work effective from 3 July 2026". I have offered my services and they have not been accepted. I have not been provided with work, and I have not asked for leave.

I do not say the Health Service may not seek medical information. It may ask. What I have not been able to identify is the provision under which, having asked, it may both decline my services and decline to pay me. If there is one, I ask to be told what it is.

If the Health Service maintains that I am not to attend, I ask that the period be treated as paid special leave under the applicable directive, rather than debited against my own accrued entitlements, given that I hold a certificate of fitness with restrictions.

I also ask that the basis recorded on 13 July 2026 be reviewed. I was advised that my leave would be submitted as "income protection no pay from the 13th July 26, as per our guidelines for all QSuper claims for Income protection". I ask which guidelines are referred to, and I record that I asked the fund to cease benefits in March 2026 because I was working. On my understanding there is no income protection claim in payment. If that understanding is wrong I would be grateful to be corrected; if it is right, I am at present receiving neither wages nor benefit.

(e) That I be provided with copies of the preserved Human Resources policies listed in Schedule 2 of the certified agreement which bear on this dispute, being at least E12 (Individual employee grievances) and E13 (Workplace Harassment). I ask for them so that I can follow whatever process applies, and I make no allegation under either.

(f) That the roster and payroll record for the twelve months preceding 3 July 2026 be provided to me, so that the pattern in fact worked is established from the Health Service's own records rather than from my estimate.

(g) That feedback be provided on the work health and safety matters I have raised, in accordance with clause 7.1.11.

(h) That a psychosocial risk assessment of the workplace be conducted in accordance with clause 7.2.

5. A MATTER I MUST RAISE ABOUT STAGE 1 ITSELF

Clause 1.11.2(a) requires that I inform my immediate supervisor. I have done so by this notice, so that there is no question that Stage 1 has been commenced.

I should record, however, that part of the subject matter of this dispute is the advice of 2 July 2026 and the reporting arrangements within Corporate Services. I have separately asked the Health Service to identify the delegate for decisions concerning my attendance and return to duty, and whether any conflict of interest has been identified and managed. I make no allegation against any individual, and clause 1.2 of the Code of Conduct for the Queensland Public

Service records that a conflict of interest is not wrongdoing in itself.

For that reason I ask the Health Service to nominate an appropriate officer to conduct the Stage 1 discussion. If it prefers that the discussion proceed with my immediate supervisor, I do not object and will attend. I raise it so that it is dealt with now rather than later, and so that no Stage 1 discussion is later said to have been conducted by the wrong person.

I ask to have a support person and my union representative present at any discussion.

6. TIME

Clause 1.11.2(a) provides that "Stage 1 discussions should take place within 24 hours, and the procedure should not extend beyond seven days". Clause 1.11.2(b) provides that if the dispute remains unresolved, it is referred to the appropriate management representative, who "shall arrange a conference of the parties", and that this process "should not extend beyond seven days". Clause 1.11.5 provides that "no party shall act in a manner unreasonably or intentionally delay the timely resolution of a dispute".

I am available for a Stage 1 discussion at any time, including today.

7. WHAT THIS NOTICE DOES NOT DO

- (a) This notice is separate from my request of the same date under clause 10.3.2 for a change in the way I work. That request is to be decided under clause 10.3.4 to 10.3.6 and is not part of this dispute.
- (b) This notice is separate from my response of the same date to the Request for Medical Information of 31 July 2026.
- (c) Clause 1.12.2 provides that the employee grievance procedure "does not limit or affect any other dispute resolution provisions contained in this Agreement". Nothing in this notice limits any other avenue, and nothing in it is a waiver of any entitlement.
- (d) This notice makes no allegation against any individual and asks for no finding to be made about anyone. It asks for the agreement to be applied.

I confirm again that I am engaging with this process and want to return to work.

Yours sincerely,

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital

0417 400 227 · coryshepherd1@hotmail.com

Conflict of interest, delegation, and the handling of my personal information

Section 89, Public Sector Act 2022 · Code of Conduct clause 1.2

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital, Metro South Health

Employee reference MSH-INJ-5795

To: Ms Noelle Cridland, Chief Executive, Metro South Health

Date: 3 August 2026

1. WHAT I AM ASKING, AND WHAT I AM NOT

I am asking that the decisions being made about my capacity, my attendance and my return to duty be made by an officer outside Corporate Services, and that the clinical material be held only by the Injury Management team.

I address this letter to you because section 89 of the Public Sector Act 2022 makes the chief executive the person to whom a conflict is disclosed, and the only person who may authorise an officer to continue to act in a matter affected by one. I do not raise it with you in order to escalate it. I raise it with you because, on the terms of the section, there is no one else who can answer the questions at Part 3.

I am not asking that anyone be removed from the management of my employment. I am not asking that anyone be stood aside, investigated, or found to have done anything wrong. I make no allegation against any individual and I seek no finding about anyone.

This is not a personal complaint. It is about how the process is being run when an employee is diagnosed with an illness and needs managerial support and reasonable adjustment.

Clause 1.2 of the Code of Conduct for the Queensland Public Service records that having a conflict of interest is not of itself wrongdoing, and that what matters is whether it is disclosed and managed. I proceed on that basis, and I ask these questions in that spirit.

2. THE POSITION AS I UNDERSTAND IT

- (a) The letters of 31 July 2026 are written in the first person of the decision-maker — "I am seeking further clarification", "I require further information", "I will make a decision regarding your ability to perform your role" — and are signed by Mr Scott Hughes, Director, Corporate Services.
- (b) Question 3 of the attached request asks my general practitioner to advise whether I am fit to return "under the existing reporting arrangements, including working with and reporting to [my] current line manager". Those reporting arrangements sit within Corporate Services.
- (c) The Manager, Switchboard Services — who on 2 July 2026 advised that my return could not be facilitated, and asked me to wait for advice as to my next returning shift — reports within that same directorate.
- (d) On 3 October 2025 I made a formal written request for review to the A/Director, Human Resources Business Partnering, concerning a process initiated by the officer who has signed the letters of 31 July 2026. My union was copied. I do not reopen that matter and I make no submission about it. I record it only because it bears on whether a conflict exists and how it must be managed.
- (e) The letter of 7 July 2026 records that my treating practitioner had advised of "unresolved workplace matters involving line management", and that the Health Service did not have sufficient information to assess whether they presented psychosocial hazards.

The officer determining my return to duty is therefore the Director of the directorate in which the advice of 2 July 2026 was given, to whom the officer who gave it reports, and whose own directorate is the subject matter of the reporting-arrangements question at question 3.

I say only that this is a set of circumstances in which a conflict may exist. Whether it does, and how it has been managed, is not something I can determine.

2A. WHAT THE "UNRESOLVED WORKPLACE MATTERS" ARE, AND WHY THEY ARE WORKPLACE MATTERS

The letter of 7 July 2026 records that the Health Service does not have sufficient information to understand the nature of the unresolved matters involving line management, or to assess whether they present psychosocial hazards. I take that at face value, and I would rather close the gap than argue about it.

The matters concern how the work is organised and decided, not the character of any officer. They are, in substance:

- (a) how patient safety concerns raised at the switchboard are escalated and answered;
- (b) how changes to the work, the roster and the on-call arrangements have been introduced, and whether affected staff were consulted before rather than told afterwards;
- (c) fatigue and recovery between shifts, and how rostering is managed against it; and
- (d) the ability of staff to have genuine input into decisions that affect the work — which the Managing the risk of psychosocial hazards at work Code of Practice 2022 treats as a matter of work design and organisational justice, and therefore as a psychosocial hazard in its own right where it is absent.

Each of those is a workplace matter. None of them can be answered by a medical practitioner writing about me. Sections 47 to 49 of the Work Health and Safety Act 2011 require the Health Service to consult workers who are directly affected when identifying hazards, when deciding how to control them, when proposing changes affecting health or safety, and when deciding the procedures for monitoring the health of workers. That consultation has not occurred at any point since 26 June 2026.

I record, without argument, that the information gap the Health Service identifies is one only the Health Service can close, and that it is being used in the meantime to maintain my exclusion.

2B. MANAGERIAL SUPPORT AND ADJUSTMENT WHILE I AM UNWELL

When an employee is certified fit with restrictions, the Health Service's own policies contemplate managerial support and reasonable adjustment: G03 clause 1.1 (temporary adjustments so an employee may "continue work during their recovery"; clause 1.1.1 extends this to a "health management case" with no compensation claim), and QH-IMP-401-5 (suitable duties identified and actively sought and "made available wherever reasonably practicable"; the worker's connection with the workplace maintained; the dispute resolution process advised — for "either a work or non-work-related injury/illness").

I obtained the checklist the Health Service required in order to facilitate my return. It certified me fit with restrictions. What followed was exclusion rather than adjustment: interim duties were declined in any capacity on 15 July 2026, and no adjustment plan has been identified to me. The Health Service may seek medical information — but a medical process clarifies functional limits; it does not displace the duty to consider adjustments in the meantime. I ask that interim adjustments be identified and implemented under the restrictions already certified.

2C. DUE PROCESS — HOW THE DECISIONS ARE MADE

Support and adjustment are the substance; due process is how the decisions about them are made: being told what power is being used and who decides; a fair opportunity to respond before a serious consequence; a decision-maker independent of the matters in issue; consultation as sections 47 to 49 of the Work Health and Safety Act require; and deadlines that are not one-sided. On the last point I record only this: I was given seven calendar days on 7 July and again on 31 July, each with a consequence attached, while the Health Service advised on 15 July that it was unable to commit to a timeframe of its own and on 30 July that it was unable to guarantee my return. I have asked several times for a date and have not been given one.

The Managing the risk of psychosocial hazards at work Code of Practice 2022 calls this organisational justice, and treats its absence — including a lack of procedural fairness — as a psychosocial hazard. I do not say any tribunal has found a denial of fair process. I say the process described has not been applied to me, and I ask that it be — which is what the separation at Part 4 is for.

3. THE QUESTIONS I ASK

Section 89(1) of the Public Sector Act 2022 provides that if a public sector employee has an interest that conflicts or may conflict with the discharge of the employee's duties, the employee:

"(a) must disclose the nature of the interest and conflict to the employee's chief executive as soon as practicable after the relevant facts come to the employee's knowledge; and (b) must not take action or further action concerning a matter that is, or may be, affected by the conflict unless authorised by the employee's chief executive."

I ask the Health Service to confirm:

- (a) whether any disclosure has been made under section 89(1)(a) in relation to the decisions concerning my attendance, my pay and my return to duty;
- (b) whether any authorisation has been given under section 89(1)(b) for an officer to take, or to continue to take, action concerning those matters;
- (c) who holds the delegation for those decisions, and the source and instrument of that delegation;
- (d) whether the advice of 2 July 2026 and the decision of 3 July 2026 were made with the knowledge or approval of the officer now determining my return to duty; and
- (e) whether any conflict has been identified and recorded, and if so, how it is being managed.

I ask these questions in writing because I have asked them before, in my email of 28 July 2026, and have not received an answer.

If no authorisation has been given under section 89(1)(b), I ask that you consider the direction contemplated by section 89(2), and that in the meantime the decisions described at Part 2 be allocated to an officer outside Corporate Services.

I am conscious that this is a request that you exercise a function under the Act, and not a request that anything be found about anyone. If the answer to each of (a) to (e) is that the matter has been dealt with, that answer will resolve it entirely and I will take it no further.

4. THE SEPARATION I PROPOSE

I propose the ordinary separation of functions. It requires no restructuring, removes no one from the management of my employment, and requires no finding to be made about anyone.

- (a) THE CLINICAL MATERIAL — held exclusively by the Injury Management team, Human Resources, on a medical-in-confidence basis. Corporate Services would not hold, and would not have access to, the clinical material. This is what the Employee Capabilities Checklist itself recommends at Additional Comments 4(c): "return-to-work coordination and correspondence through the Injury Management team, Human Resources."
- (b) THE DECISION — any decision concerning my capacity or my return to duty made by an officer outside Corporate Services.
- (c) IMPLEMENTATION — Corporate Services retains the operational role: rostering me, putting in place whatever adjustments are decided, and the ordinary day-to-day management of my work. My line manager would receive the adjustments to be implemented, not the clinical information behind them.

The officer steps out of the decision, not out of the workplace.

Metro South Health is a large organisation and I do not anticipate that this would present any difficulty.

5. MY PERSONAL INFORMATION

The information sought by the request of 31 July 2026 is sensitive health information about me. I ask:

- (a) that it be held by the Injury Management team, used only for the purpose for which it was collected, and not provided to Corporate Services — what Corporate Services requires in order to roster me and implement adjustments is the adjustments themselves, not the diagnosis or the clinical reasoning behind them;
- (b) to be told who has had access to the medical material already held about me, who will have access to the further material, and whether it is proposed to provide any of it to any person outside the Health Service, and for what purpose; and
- (c) that it not be used in connection with any other proceeding. I do not consent to that.

I make these requests under the Information Privacy Act 2009 and the Queensland Privacy Principles, and consistently with the consent I have given, which is scoped in my response of the same date.

6. WHY IT MATTERS TO THE DECISION ITSELF

I raise one further matter, neutrally.

A decision on my request for a change in the way I work may only be made, or refused, on reasonable grounds. Whether grounds are reasonable is not only a question of what was decided; it is also a question of who decided it and

on what material. If the decision is made by an officer within the arrangements the decision concerns, that is a matter which can be raised later, by either of us, and I would rather it were dealt with now.

I would also note that the Managing the risk of psychosocial hazards at work Code of Practice 2022 identifies poor organisational justice — including a lack of procedural fairness — as a psychosocial hazard in its own right, and that HR Policy G03 requires proper consideration to be given to that Code.

I therefore ask that this be resolved before the decisions are made, rather than after.

6.1 ONE MATTER I RECORD FOR THE FILE

I record the following once, neutrally, and I do not develop it here.

In the course of my employment I have raised concerns about patient safety, about changes to the work and the roster, about fatigue, and about the ability of staff to have input into decisions affecting the work. I am entitled to raise those matters, and to be consulted on work health and safety matters that affect me.

Since 3 July 2026 I have been held out of the workplace, my accrued leave has been debited, and from 13 July 2026 I have been without wages, while those matters remain unassessed by the Health Service and while the decisions about my return sit within the directorate they concern. That is a serious detriment to me.

I am not making a claim in this letter and I ask for no finding. I record it because it is a matter I have raised with my union, and because I would rather it be dealt with by the Health Service now than become an issue later. I do not abandon my employment or any entitlement.

7. WHAT THIS LETTER DOES NOT DO

- (a) It makes no allegation against any individual and asks for no finding about anyone.
- (b) It does not ask that anyone be removed from the workplace, stood aside or investigated.
- (c) It is separate from my response of the same date to the Request for Medical Information, from my request under clause 10.3.2, and from my notice of dispute under clause 1.11. A decision on any of those is not an answer to this.
- (d) It is not a complaint under any Act, and it is not a grievance.

I confirm again that I am engaging with this process and want to return to work.

8. WHAT I ENCLOSE

I enclose only what is necessary to identify the matters referred to above. I have not attached the wider correspondence, and I am content to provide anything further on request.

- (a) The signature page of the Request for Medical Information of 31 July 2026.
- (b) The extract of question 3 of that request.
- (c) The relevant page of the Employee Capability Checklist of 3 July 2026.
- (d) The email of 2 July 2026 concerning the Employee Capacity Certificate and my return.

Yours sincerely,

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital

0417 400 227 · coryshepherd1@hotmail.com